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The rural interdisciplinary team training program: a workforce development workshop to increase geriatrics knowledge and skills for rural providers

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ABSTRACT

The Rural Interdisciplinary Team Training Program (RITT) is a team-based educational component of the Veterans Health Administration (VHA) Office of Rural Health Geriatric Scholars Program. It is a workforce development program to enhance the geriatrics knowledge and skills of VA primary care clinicians and staff caring for older veterans in rural communities. The RITT workshop, accredited for 6.5 hours, is interactive and multi-modal with didactic mini-lectures, interactive case discussions and role play demonstrations of assessments. Clinic teams also develop and implement a small quality improvement project based on common challenges faced by older persons. This report is an evaluation of the effect of the RITT Program on geriatrics knowledge and team development as well as success in developing and implementing the quality improvement projects in 80 VHA rural outpatient clinics in 38 states.

KEYWORDS

Teamwork; rural; interdisciplinary education; continuing medical education; geriatric education/training

Three million veterans live in rural communities (https://www.ruralhealth.va.gov/) where access to geriatrics specialists is scarce or non-existent (Tumosa et al., 2012). This is important because 42% of veterans are 65 years of age and older and nearly 1.2 million are 85 years of age and older, with more than half of the veterans seen in primary care services each year being 65 years of age and older (Shay, 2015). The Rural Interdisciplinary Team Training Program (RITT) provides specialized geriatric knowledge to Veterans Affairs (VA) primary care teams working in rural VA Community-Based Outpatient Clinics (CBOCs), which are often closer to veterans' homes than tertiary VA medical centers where most VA specialists are located. Appendix C contains a complete list of acronyms. RITT is a team-based educational component of the Veterans Health Administration (VHA) Office of Rural Health Geriatric Scholars Program, a continuing professional development program to enhance the knowledge and skills of VA primary care clinicians and staff in caring for older veterans (B. Josea Kramer et al., 2016). As of September 30, 2016 the RITT program had trained primary care clinics in each of the 18 regional Veteran Integrated Service Networks and reached into 35 states. This

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report describes the RITT program and evaluates its three components: team development improvement, acquisition of geriatrics knowledge and success in implementing an action plan, a small quality improvement project.

Created in 2011, RITT is an interactive, multi-modal workshop drawing on the principles of adult learning (Knowles, 1980) which aims to strengthen team-based solutions in recognizing issues faced by older veterans. The workshop format was selected because this approach is focused, intensive, and addresses specific work-related or organizational issues (Klatt, 1999). The RITT program is offered to all Geriatric Scholars during their program orientation and the RITT coordinator also reaches out to CBOC leaders. RITT is available to VA employees regardless of prior participation in the Geriatric Scholars Program. Participants are recruited locally by CBOC points of contact who are usually clinic administrators or nurse managers. Recommended RITT participants include all members from the primary care team, such as physicians, nurses, social workers, psychologists, pharmacists, rehabilitative medicine and administrative staff. In addition, teams frequently include members from other disciplines, such as occupational therapy, physical therapy, and nutrition services. The curriculum, which is outlined in Table 1 and accredited for 6.5 hours of continuing education credits includes didactic mini-lectures, interactive discussions based on case studies and video presentations, role play demonstrations of assessments and the development of an action plan – which is a small quality improvement project – to improve care of older veterans based on the information presented in the workshop. RITT is delivered on-site at the CBOCs by two faculty members; a clinical specialist in geriatrics with a background in medicine, nursing or psychology as well as an education specialist. A team teaching format draws on each of the trainer's strengths, contributes to a more interactive and engaging day, and enhances participants' learning (Anderson et al., 1996). The trainers for RITT are recruited from VHA Geriatrics Research, Education and Clinical Centers which are centers of excellence in geriatrics. RITT faculty follow a standardized slide set and related learning objectives. New faculty receive 1.5 hours of training on the curriculum and are asked to observe a workshop to better understand the content, delivery methods and strategies for participant engagement. The curriculum is reviewed annually by two to four geriatrics subject matter experts, with trainers offering suggestions for modification as well.

RITT expanded its target audience to Indian Health Services (IHS) and tribal clinics in 2016 under the authority of a VHA and IHS Memorandum of Understanding to improve the health status of American Indian and Alaskan Native Veterans. The evaluation results reported in this paper do not include findings from these IHS and tribal clinics.

Table 1. RITT curriculum.

Competencies	Teaching strategies
Articulating the four components of an effective interdisciplinary team	Brief didactic presentation Video case with discussion
Describing four common geriatric syndromes	Didactic presentation Trainer role play demonstrating how to use a cane and gait assessment
Applying three team based solutions to care for older Veterans	Unfolding case discussion focusing on team based transitions
Differentiating the clinical applications of cognitive assessment tools	Trainer role play demonstrating cognitive assessment
Developing a team based Action Plan for practice change	Brief didactic presentation on the principles of quality improvement Group discussion leading to the identification of an Action Plan and steps/resources needed for implementation

Evaluation methods

Between 2012 and 2016, RITT training was conducted at 80 unique rural VHA CBOCS. Over the four years of the evaluation, 1,159 VA employees participated in RITT workshops. The Institutional Review Board based at VA Greater Los Angeles Healthcare System granted the project an exemption from human subjects review. The evaluation strategy was a mixed-methods approach that included both quantitative and qualitative analyses. There were four components to our evaluation. First, we evaluated team development with a web-based survey administered before the workshop and at three and six months after the workshop. Second, we evaluated change in geriatric knowledge from immediately before to immediately after the workshop with geriatrics knowledge test that included multiple choice and true/false questions. Third, we used content analysis to evaluate the quality of action plans. In addition, we evaluated participant satisfaction with the program. Specific measures and analyses are described below.

Team development measure

We measured team development by asking individual team members to complete the Team Development Measure (TDM) (Stock, Mahoney, & Carney, 2013) using a web-based survey instrument before the workshop and again at three and/or six months after the workshop. The TDM has been used to assess teams of various sizes and backgrounds, including administrative healthcare teams and inpatient and outpatient staff (Stock et al., 2013). It is used as a tool for evaluating team dynamics and overall team performance. The TDM is shown in Appendix A. The TDM questionnaire has 31 items that together measure four key features of teams: (1) cohesiveness, (2) role clarity, (3) communication and (4) clarity of goals and means. The TDM is scored using a four-point Likert scale with the answer choices set at "disagree strongly", "disagree", "agree" and "strongly agree". The overall score is calculated by summing the responses (with disagree strongly = 1 point to strongly agree = 4 points) for a maximum raw score of 124 with a higher score indicating better overall team performance. The raw score is then converted to a 100-point scale with predefined instructions on conversion (Salem-Schatz, Ordin, & Mittman, 2010). There were 928 participants who completed the TDM at baseline, 173 at three months and 263 at six months. Because the majority of respondents did not complete the TDM at multiple time points we were only able to match 219 of 928 pre-workshop scores to a follow-up score. In order to maximize our sample size, we used an independent sample t-test to compare scores before (n = 928) and at follow-up (n = 436).

A qualitative text analysis was applied to two open-ended questions that were added to the TDM: (1) "What do you think is the biggest challenge for your team when providing care for older adults?" and (2) "What topic areas does your team need additional training on in order to improve the quality of care provided to older adults?" Three authors read the responses of participants to the open-ended questions in groups of two. We developed content codes with new categories of content until saturation (Hsieh & Shannon, 2005). We read random samples of the coding of the other two coders and discussed results to achieve consensus. We resolved disagreements by discussing the reasons each coder had for categories until we all agreed. We reviewed the participants' answers at two time

periods: before the workshop and at follow-up. This yielded 1597 responses to the questions from 909 individuals. We coded content for each question separately.

Geriatrics knowledge test

As part of the annual review of the RITT curriculum and teaching methods, the geriatrics knowledge test was instituted in 2015. We developed the questions to track with the curriculum and allow us to assess changes in participants' geriatrics knowledge before and after the workshop. These questions were reviewed by two board certified geriatricians for medical accuracy. The geriatric knowledge test has 15 multiple choice and true/false questions that cover the subject matter and can be found in Appendix B. The test is scored as the number of correct responses of 15 items multiplied by 100. The geriatrics knowledge test was administered both immediately before and immediately after the workshop, on the same day as the workshop. Participants created a unique identifier to use on both their pre- and post-tests. Individual scores were then matched. In a few cases, participants did not complete both the pre- and post-test, so we did not include those in the analysis. We used a paired samples t-test to compare scores pre- and post-test.

Action plans

The workshop culminates with the development of action plans to carry forward the knowledge and skills learned during the workshop in the context of the local needs of the CBOC. These action plans are based on the Plan, Do, Study, Act model which is widely used in healthcare settings. This model is an important tool to assist clinic teams in focusing on an area of quality improvement and testing this plan on a small and manageable scale ("Plan-Do-Study-Act Cycle," 2008). Action plan topics are derived from the Assessing Care of Vulnerable Elders measures (Wenger & Shekelle, 2001) for: (1) cognitive assessment; (2) falls prevention; (3) depression; (4) medication reconciliation; (5) urinary incontinence. Teams also had the option to develop a locally generated plan addressing a problem relevant to their clinic. RITT trainers worked with teams to identify potential pitfalls in implementation and methods to reduce obstacles. The clinic staff designated a point person for the RITT coordinator to contact post-training to assess team progress in implementation. The implementation of the action plan is monitored by RITT education staff at approximately three months after the training by a telephone call with the point of contact. In the earlier years of RITT (2012–2014), follow-up on action plans was performed once between six and 12 months post-workshop. However, in more recent years (2015–2016), follow-up was done at one and/or three months post-workshop. The difference in time to follow-up on action plans reflects differences in the project coordinators over the four years of the project. If the point of contact could not be reached after three attempts at either time point, the RITT coordinator attempted to reach other team members from the CBOC. For our analyses, we measured follow-up at any time point at which data were available.

We evaluated the content and implementation of the action plans using qualitative analyses. Two members of the evaluation team read through all of the action plans developed between 2012 and 2016. We used descriptive content analysis to develop

topic categories based on the specific aim statement of the action plan until saturation (Hsieh & Shannon, 2005).

Results

As indicated in Table 2, trainees from a nursing discipline made up the largest category of trainees (46.7%). Participants identified as health technicians or health clerks comprised the second largest category (17.1%). Clerks and health technicians are important for the smooth management of the clinic. In particular, they are the first people to see the patient at check in. They often know patients by sight. They facilitate the flow of patients, answer and direct calls, and implement medication refill requests. Physicians made up the third largest category (14.1%) and social workers comprised the fourth largest category (7.8%).

TDM

Respondents completed the TDM online before (n = 928) and 3–6 months after the workshop (n = 436). The results from the independent samples t-test are as follows. The average TDM score increased from before the workshop (mean = 55.3, standard deviation [SD] = 8.6) to 3–6 months after the workshop (mean = 57.2, SD = 8.3) (p = .001).

Table 2. Descriptive statistics professional backgrounds of RITT trainees (n = 1159).

Disciplines and job titles	*N* (%)
Medicine	
Medical Doctor/Doctor of Osteopathic Medicine	147 (12.7)
Physician Assistant	16 (1.4)
Nursing	
RNs	302 (26.1)
LVN/LPN	187 (16.1)
Nurse Practitioner	47 (4.1)
APRN, FPRN	4 (0.3)
Nursing Aide/Nursing Assistant	1 (0.1)
Ancillary services	
Social Work	90 (7.8)
Pharmacy	35 (3.0)
Psychology	34 (2.9)
Nutrition	25 (2.2)
Rehabilitation (OT/OTA/PT/KT)	22 (1.9)
Dental	1 (0.1)
Administrative	
Health Tech/Health Clerk	189 (16.4)
Clinic Administration	8 (0.7)
Other (e.g. Exercise specialists, VA Office of Tribal Government Relations)	51 (4.4)

Percentages do not sum to 100% due to rounding.
Abbreviations
APRN: Advanced Practice Registered Nurse
FPRN: Family Practice Registered Nurse
GNP: Geriatric Nurse Practitioner
LVN: Licensed Vocational Nurse
LPN: Licensed Practice Nurse
OT: Occupational Therapy
OTA: Occupational Therapy Assistant
PT: Physical Therapy
KT: Kinesiology Therapist

Although this 2% increase was statistically significant, it is unlikely to reflect a meaningful improvement in teamwork dynamics.

Participants cited a number of challenges in working with older veterans, including VHA resources and services, VHA entitlements (e.g. space, partnership with non-VHA program) and time constraints (Table 3). Participants also identified a need for more education on VHA service and programs, team communication, and improving patient care (Table 4).

Geriatrics knowledge test

Scores on the geriatric knowledge test were significantly higher (11.5 points higher) at post-test than at pre-test (n = 250, t = 15.2, df = 1, p < .001). There is no evidence of floor or ceiling effects in the pre- training data because the scores have a normal distribution. See Table 5.

Table 3. Descriptive statistics for answers to question 1before and after workshop.

	Before workshop	After workshop
	Number of responses (%)	
Organization structure		
VA resources and services (e.g. VA entitlements, space, non-VA partnerships)	110 (18)	57 (15)
Staffing	108 (18)	55 (15)
Time constraints	102 (17)	67 (18)
Team process		
Staff communication	56 (9)	25 (7)
Team roles/responsibilities	46 (8)	34 (9)
Staff education about geriatrics	14 (2)	4 (1)
Patient		
Medical condition	59 (10)	35 (9)
Continuity of care/Care coordination	35 (6)	17 (5)
Patient or community lack of resources	32 (5)	46 (12)
Behavior	28 (5)	25 (7)
Medications/polypharmacy	10 (2)	9 (2)
Safety	3 (.5)	1 (0.3)
Falls	2 (.3)	0 (0)
	N = 605 responses	N = 375 responses

Table 4. Descriptive statistics for answers to question 2 before and after workshop.

	Before workshop	After workshop
	Number of responses (%)	
Topics for more training		
VA resources and services	131 (30)	52 (28)
Communication/Teamwork	106 (24)	50 (27)
Improving patient care	62 (14)	24 (13)
Patient's medical condition	36 (8)	13 (7)
Staff education about geriatrics	35 (8)	12 (7)
Mental health	18 (4)	6 (3)
Dementia	17 (4)	9 (5)
Family caregivers	16 (4)	10 (5)
Medications/polypharmacy	13 (3)	7 (4)
	n = 434	n = 183

Table 5. Results of t-test and descriptive statistics for geriatric knowledge test before and after workshop.

	Average score before workshop		Average score after workshop			Mean difference		
Outcome	M	SD	M	SD	*n*		*t*	*df*
	68.2	11.7	79.7	10.8	250	11.5	15.2*	1

*$p = .0001$

Note: The number of trainees for this test is smaller than total trainees because the test was not implemented until 2015.

Action plans

Workshop participants developed 139 action plans during the evaluation period (Table 6). The total number of plans is greater than the number of sites because several clinics divided their participants into smaller workgroups with separate action plans. Seventy-one (51%) plans were in progress when surveyed at either one and/or three months post-training. In 2015 and 2016, 15 action plans had follow-up data at one time point (one or three months post-training) and 21 plans had follow-up data at two time points (both one and three months post-training). While enthusiasm was generally high for carrying out these plans at the end of the workshop, just over half (51%) of the 139 action plans were implemented or in progress at follow-up. There were a number of barriers cited for difficulties in implementation, with the most often cited being staff turnover, lack of resources (time and staff) and lack of endorsement by leadership at the parent facility above the level of the patient aligned care teams.

Participant satisfaction

Participant satisfaction was measured by the accreditation sponsor – the VHA Employee Education System – within 30 days of the conclusion of the course through a web-based program

Table 6. Descriptive statistics for action plan topics.

Action plan topic	*n* (%)
Assessment of cognitive impairment	26 (18.7)
Medication reconciliation	23 (16.5)
Patient Education	12 (8.6)
Care coordination/Transitions of care	11 (7.9)
Improvement of clinic processes/Appointments	9 (6.5)
Routine team meetings about patients	8 (5.8)
Falls reduction	7 (5.0)
Increased staff education	7 (5.0)
Identification of red flags/Geriatric syndromes	5 (3.6)
Review of clinic processes and Other clinic issues	5 (3.6)
Assessment of mental health/Depression	4 (2.9)
Assessment for diabetes	4 (2.9)
Review of clinic processes for medication review	3 (2.2)
Improving teamwork/Communication	2 (1.4)
Improving safety outcomes for patients	2 (1.4)
Getting Health Insurance Portability and Accountability Act Forms signed	2 (1.4)
One-of-a-kind topics	
Decreasing no shows	1 (0.7)
Assessment clinic processes for laboratory tests	1 (0.7)
Increasing geriatric assessments	1 (0.7)
Increasing driving assessments	1 (0.7)
Enhancing coordination with non-VA care	1 (0.7)
Increasing use of tele-buddies	1 (0.7)
Developing a template to find care for patients' pets	1 (0.7)
Ensuring that patients have smoke detectors	1 (0.7)
Assessing home oxygen needs of patients	1 (0.7)
	n = 139

assessing satisfaction on a five-point scale. For this scale, a score of 1 indicates strongly disagree and 5 indicates strongly agree. RITT participants indicated that they were highly satisfied with the RITT workshops. For 2015 and 2016, 90% of people indicated that they agreed or strongly agreed with the statement "Overall, I was satisfied with this learning activity". Additionally, 89% of persons in 2015 and 2016 indicated that they agreed or strongly agreed with the statement "The scope of the learning activity was appropriate to my professional needs".

Table 7 shows the results of a post-hoc review of action plans that examine the relationship between site identification of staff turnover as a concern and whether the action plans were implemented. The majority of sites (n = 104) did not implement their action plans. Out of the 25 sites that partially or fully implemented the action plans, one or more staff members identified staff turnover as a contributing factor (32% versus 6% among the sites that did not implement). Eight sites that implemented the action plans also noted staff turn as a positive factor in implementation because turnover brought new team members who supported the action plans. Other sites indicated that turnover can be a negative factor, e.g. "Everyone associated with the plan is gone".

Discussion

We implemented and evaluated a team-based workshop to enhance geriatrics knowledge and skills in 80 rural VA CBOCs between 2012 and 2016. Participation in this program was associated with a significant short-term increase in geriatrics knowledge as measured by a pre- post- test.

There was a modest improvement in teamwork as reported on the Team Development Measure. The increase in teamwork was statistically significant but too small to be meaningful. It is not surprising that a one-day workshop was not enough to change team development. There was insufficient post-workshop reinforcement of the training. Moreover, periodic mentoring, brief educational sessions, and positive feedback for improved teamwork are all needed to change team dynamics (Weaver, Dy & Rosen, 2014).

The teams also developed action plans, or small quality improvement projects, aimed at improving processes and outcomes of care; however, nearly half were not initiated at three months. We explored possible reasons for this finding and conducted a post-hoc review of action plans, examining the relationship between clinic identification of staff turnover as a concern and action plan implementation. A team initiative similar to the RITT training found that staff changes disrupted team dynamics (Kuder, Gairola, & Hamilton, 2001) and we surmised that this might have contributed to the lack of implementation. However, surprisingly, we found that more than half of our small sample cited staff turnover as a positive factor in implementation due to the hiring of new staff who championed the action plan. While it is impossible to identify the reasons for this, it may well be that staff members who left the clinics were burned out and overwhelmed and not enthusiastic

Table 7. Results of chi-square test and descriptive statistics for action plan implementation by discussion of staff turnover.

Staff	Action plan implementation	
Turnover	Not-implemented	Implemented
No	98 (94%)	17 (68%)
Yes	6 (6%)	8 (32%)

Note. X^2 = 14.33, df = 1. Numbers in parentheses indicate column percentages.
*$p < .05$

about taking on new projects such as implementing the action plans. High staff turnover is common in rural healthcare settings because of lack of resources, professional isolation, and heavy and challenging workloads, professional and personal burnout and rural lifestyle challenges (Chipp et al., 2011). *Locum tenens* staff with relatively short time commitments are frequently the only option for providing healthcare. However, it is our observation that most rural clinics have long-term non-physician core staff that may well be the "engine" of the clinic and persevere regardless of provider turnover.

Limitations

A limitation of this study was the inability to obtain follow-up information from the clinic contacts regarding the action plans. The RITT coordinator made multiple attempts to contact the designated point person about the action plan and its initiation. However, repeated attempts for follow-up were often unsuccessful. Even when follow-up information was available, many clinics had not implemented their action plans. Loss to follow-up is a common challenge in longitudinal studies. One potential limitation regarding the lack of success of the implementation of action plans is that the action plan topics reflect the needs of clinic administration (i.e. top down) rather than the needs identified by RITT participants. However, the content of the action plans was team driven and not directed by clinic leadership. Another limitation is that the questions on the geriatric knowledge test were generated by expert opinion and aside from content validity these questions were not otherwise validated. Thus, there is a chance that improved scores could be from a testing effect. A final limitation is that knowledge acquisition was tested immediately after the workshop and not at a later point in time so that a long-term association with increased geriatrics knowledge is unknown.

Conclusion

Now in its seventh year, the RITT program is established and poised to move into a broader dissemination phase to non-VA clinics, including IHS/Tribal Health clinics and clinics sponsored by Alaska Native corporations. Discussions are currently underway with additional non-VA partners such as the Health Resources and Service Administration Geriatric Workforce Enhancement Program grantees to adapt RITT in non-VA settings. The program is laying the groundwork for an active dissemination strategy including faculty development seminars and a replication guide. One concern has been the low rate of completion of the action plans and implications for the dissemination phase. We plan to address this by more frequent post-workshop follow-up through e-mails with embedded electronic survey links to obtain information and a webinar about strategies for successful action plan implementation. In addition to more intensive follow-up regarding action plans, we will host quarterly webinars focusing on topics covered in the RITT curriculum (e.g. assessment tools, common geriatric conditions). These resources will be offered to healthcare clinics and trainers in non-VA networks who agree to comply with fidelity, information collection and evaluation protocols.

Disclosure statement

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Appendix A

The Team Development Measure

This questionnaire is a measure of team characteristics. Please indicate how much you strongly disagree – disagree – agree – strongly agree to each statement as it applies to your team at the present time. There are no right or wrong answers, just your perceptions. This survey is totally anonymous.

Before beginning please write the name of your team on this line

	Disagree Strongly	Disagree	Agree	Agree Strongly
1. Team members say what they really mean.	DS	D	A	AS
2. Team members say what they really think.	DS	D	A	AS
3. Team members talk about other team members behind their back.	DS	D	A	AS
4. All team members participate in making decisions about the work of the team.	DS	D	A	AS
5. All team members feel free to share their ideas with the team.	DS	D	A	AS
6. All team members feel free to express their feelings with the team.	DS	D	A	AS
7. The team practices tolerance flexibility and appreciation of the unique differences between team members.	DS	D	A	AS
8. The team handles conflicts in a calm caring and healing manner.	DS	D	A	AS
9. Regardless of the topic communication between the people on this team is direct, truthful, respectful and positive.	DS	D	A	AS
10. The team openly discusses decisions that affect the work of the team before they are made.	DS	D	A	AS
11. In this team, members support, nurture and care for each other.	DS	D	A	AS
12. The team has agreed upon clear criteria for evaluating the outcomes of the team's effort.	DS	D	A	AS
13. As a team we come up with creative solutions to problems.	DS	D	A	AS
14. In the team there is more of a WE feeling than a ME feeling.	DS	D	A	AS
15. There is confusion about what the work is that the team should be doing.	DS	D	A	AS
16. There is confusion about how to accomplish the work of the team.	DS	D	A	AS
17. Roles and responsibilities of individual team members are clearly understood by all members of the team.	DS	D	A	AS
18. All team members place the accomplishments of the team ahead of their own individual accomplishments.	DS	D	A	AS
19. The goals of the team are clearly understood by all team members.	DS	D	A	AS
20. All team members define the goals of the team as more important than their own personal goals.	DS	D	A	AS
21. I am happy with the outcomes of the team's work so far. .	DS	D	A	AS
22. I enjoy being in the company of the other members of the team.	DS	D	A	AS
23. This team is a personally meaningful experience for me..	DS	D	A	AS
24. I have a clear understanding of what other team members expect of me as a team member.	DS	D	A	AS
25. The work I do on this team is valued by the other team members.	DS	D	A	AS
26. I am allowed to use my unique personal skills and abilities for the benefit of the team.	DS	D	A	AS
27. Some members of this team resist being led.	DS	D	A	AS
28. Information that is important for the team to have is openly shared by and with all team members.	DS	D	A	AS
29. All individuals on this team feel free to suggest ways to improve how the team functions.	DS	D	A	AS
30. When team problems arise the team openly explores options to solve them.	DS	D	A	AS
31. On this team the person who takes the lead differs depending on who is best suited for the task.	DS	D	A	AS

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Appendix B

Geriatrics Knowledge PRE* Assessment
CBOC Location _____
Identifier Nickname for Pre and Post Assessments _____
True or False

1. TRUE/FALSE	More than half of the Veterans seen in primary care each year are aged 65 years and older.
2. TRUE/FALSE	Urban older Veterans are more likely than their rural counterparts to have difficulties with activities of daily living (ADLs)
3. TRUE/FALSE	The proper way to use a cane is to hold it on the same side of the body as the problem leg
4. TRUE/FALSE	Among Americans aged 65 years and older, one out of four persons have multiple chronic conditions
5. TRUE/FALSE	Mr. Robertson is an 81-year-old Veteran who reports frequent occasions of getting lost while driving to otherwise familiar places, but has no other observable cognitive deficits. This criteria would indicate dementia.
6. TRUE/FALSE	Team effectiveness is associated with better patient outcomes.

Multiple Choice

7. All of the following are examples of universal/common changes as people age, EXCEPT:
   (a) Decreased night vision
   (b) Decreased memory function
   (c) Decreased lung vital capacity
   (d) Decreased gait speed
8. How far should a patient walk away from the chair when performing a Timed Get Up and Go Test?
   (a) 5 feet
   (b) 10 feet
   (c) 15 feet
   (d) 20 feet
9. All of the following are red flags of Alzheimer's Disease EXCEPT:
   (a) Difficulty completing familiar tasks at home
   (b) Trouble understanding spatial relationships
   (c) Losing things from time to time
   (d) New problems with words when speaking
   (e) Withdrawal from work or social activities
10. Deficits in Instrumental Activities of Daily Living (IADLs) can indicate the possible presence of dementia. Which of the following are IADLs?
   (a) Managing finances
   (b) Personal hygiene
   (c) Managing medications
   (d) Feeding
   (e) A and C
11. During a consultation with an 85-year-old man with a history of frequent falls, you might recommend all of the following home modifications EXCEPT:
   (a) Remove throw rugs
   (b) Use of rubber mats in the bath tub
   (c) Install hand rails on entry way stairs

(d) Use of low-lying furniture
(e) Clear furniture from common paths to the bathroom
(f) Use of slippers with rubber soles

12. A 76-year-old male Veteran with diabetes is diagnosed with glaucoma. What challenges may the patient face in monitoring and treating his diabetes?
(a) Difficulty identifying medications
(b) Decreased balance may cause difficulties in exercising
(c) Negative medication interactions
(d) More challenging to interpret insulin pen
(e) All of the above
13. What are some of the benefits of teamwork in primary care?
(a) Decrease duplication of services
(b) Assists in transitions of care
(c) Promotes creative solutions to complex problems
(d) Distributes workload in overburdened practices
(e) All of the above
14. Poor transitions of care can lead to all of the following EXCEPT:
(a) Medication errors
(b) Increased care costs
(c) Increased caregiver stress
(d) Increased patient understanding
15. All of the following are required for efficient information exchange EXCEPT:
(a) Occasional ad hoc unscheduled meetings
(b) Clear team roles
(c) Agreed upon goals
(d) Opportunities for team discussion

* The post-assessment is identical to the pre-assessment with questions in a different order.

Appendix C

Acronyms in the body of the paper
Community Based Outpatient Clinics (CBOCs)
Indian Health Services (IHS)
Team Development Measure (TDM)
The Rural Interdisciplinary Team Training Program (RITT)
Veteran Affairs (VA)
Veterans Health Administration (VHA)